

REGULATORY DOSSIER SUBMISSION

Dossier ID: DOS-0854B36BD2

Country: Uganda

Submission Date: 2025-08-21

Product: Cefpra (cefprozime) | ATC: J01DD13 | Strength: 500 mg

Applicant: Global Therapeutics | Manufacturer: Kalahari Generics

Policy Label: standard_review | Risk Score: 0.5

AMR Stewardship: AwaRe=watch | Unmet Need=routine | GLASS Trend=declining | Watch
Similarity=moderate

SECTION CONTENT

[1] m1_application_admin - Application Form and Administrative Information

Labels: presence=present; length=length_ok; correctness=correct

This dossier constitutes a formal application for Marketing Authorization of Cefpra (cefprozime) submitted on 2025-08-21 to the national regulatory authority of Uganda. The applicant, Global Therapeutics, formally requests approval for the commercial distribution of this medicinal product. The proposed pharmaceutical form is tablet with a strength of 500 mg, classified under ATC code J01DD13. All required administrative declarations, legal attestations, letters of authorization, and regional application forms have been completed, signed by the responsible Qualified Person, and appended to Module 1. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Method validation records and quality controls were reviewed for internal consistency. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. A randomized, double-blind, parallel-group study design was employed to minimize selection and ob

[1] m1_manufacturer_gmp - Manufacturer and GMP Evidence

Labels: presence=present; length=length_ok; correctness=correct

The primary commercial manufacturing site for the finished pharmaceutical product is Kalahari Generics, located in Botswana. The facility was subject to a comprehensive GMP inspection by a recognized stringent regulatory authority. The latest inspection concluded on 2025-11-30, resulting in a formal compliance status of 'compliant'. The active GMP certificate number is GMP-970914, which remains valid until 2028-05-28. Site master files, a summary of recent inspection observations, and evidence of completed Corrective and Preventive Actions (CAPA) are included in the annex. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Method validation records and quality controls were reviewed for internal consistency. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Site quality-system commitments were compared against the proposed commercial manufacturing activities. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. All referenced documents are available in the dossier annex and traceable by section identifier. Batch manufacturing records and standard operating procedures (SOPs) are maintained and archived on-site. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The

manufact

Site Inspection History Summary:

Inspection Date	Authority	Scope	Outcome
2025-11-30	National Auth	FPP/General	compliant
2022-05-12	SRA Joint	API/Sterile	compliant

[1] m1_product_information - Product Information and Naming

Labels: presence=present; length=length_ok; correctness=correct

The proposed proprietary name for this medicinal product is Cefpra, containing the active pharmaceutical ingredient cefpodoxime. The draft Summary of Product Characteristics (SmPC), Patient Information Leaflet (PIL), and primary/secondary packaging labels are provided for review. These documents detail the approved therapeutic indications, posology, contraindications, special warnings, and precautions for use. A comprehensive risk management plan to minimize medication errors is also submitted. WHO AWaRe category: Watch. Similarity to existing Watch comparator ceftriaxone is assessed as moderate. Stewardship materials describe controlled use to limit cross-resistance press

[2] m2_quality_overall_summary - Quality Overall Summary

Labels: presence=present; length=length_ok; correctness=correct

This Quality Overall Summary (QOS) provides a critical evaluation of the chemistry, manufacturing, and controls (CMC) data for cefpodoxime. The control strategy justifies the proposed specifications for both the active substance and the finished product, emphasizing critical quality attributes (CQAs) and critical process parameters (CPPs). Impurity profiles, including degradation products and potential genotoxic impurities, have been thoroughly characterized. Batch analysis data from three commercial-scale validation batches demonstrate consistent manufacturing performance and compliance with all release criteria. Method validation records and quality controls were reviewed for internal consistency. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The applicant confirms alignment with applicable technical guidance and regional submission standards. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias.

[2] m2_clinical_overview - Clinical Overview and Benefit-Risk Summary

Labels: presence=present; length=length_ok; correctness=correct

The Clinical Overview synthesizes the efficacy and safety data supporting the use of Cefpra for the treatment of moderate pain. The pivotal clinical development program comprised 4 well-controlled studies. Based on the integrated analysis, the reported clinical outcome category is 'endpoint_met'. The overall benefit-risk profile is considered highly favorable for the target patient population. Safety findings were consistent with the known pharmacological class effects, and appropriate risk minimization measures have been integrated into the proposed prescribing information. GLASS-aligned surveillance trend is declining, and cross-resistance risk relative to ceftriaxone is assessed as moderate. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The risk management

plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The clinical overview links the therapeutic rationale to the pivotal evidence package and the proposed indication. Method validation records and quality controls were reviewed for internal consistency. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Stability testing was conducted in climatic

[3] m3_api_quality - API Quality and Control Strategy

Labels: presence=present; length=length_ok; correctness=correct

The active pharmaceutical ingredient (API) section details the complete synthesis route, starting from well-defined regulatory starting materials. Robust in-process controls and intermediate specifications ensure the consistent quality of the final drug substance. The analytical procedures used for release and stability testing have been fully validated for accuracy, precision, linearity, and robustness. The impurity control strategy adequately addresses organic impurities, residual solvents, and elemental impurities in accordance with current ICH guidelines. Forced degradation studies confirm the stability-indicating nature of the assay methods. The API package links specification limits to validated analytical methods and impurity-control decisions. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Critical material attributes and control strategy elements were reviewed for consistency across the quality dossier. Method validation records and quality controls were reviewed for internal consistency. The applicant confirms alignment with applicable technical guidance and regional submission standards. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. All referenced documents are available in the dossier annex and traceable by section identifier. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Residual solvents and elemental impurities are controlled well below ICH Q3C and Q3D permissible daily exposures. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. The synthetic route involves three well-characterized steps with defined starting materials and isolated intermediates. Batch analysis, validation, and impurity management records support the proposed API quality framework. Adverse eve

[3] m3_fpp_manufacturing - FPP Manufacturing Process and Controls

Labels: presence=present; length=length_ok; correctness=correct

The finished pharmaceutical product (FPP) manufacturing process utilizes standard, scalable unit operations. A formal quality risk assessment (e.g., FMEA) was conducted to identify critical process parameters (CPPs) that impact critical quality attributes (CQAs). Process performance qualification (PPQ) reports for three consecutive commercial-scale batches verify that the process is maintained in a state of control. In-process controls, intermediate hold-time studies, and packaging validation data are presented to substantiate the robustness and

reproducibility of the commercial manufacturing operations. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Method validation records and quality controls were reviewed for internal consistency. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. All referenced documents are available in the dossier annex and traceable by section identifier. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The applicant confirms alignment with applicable technical guidance and regional submissi

[3] m3_stability - Stability and Shelf-Life Justification

Labels: presence=present; length=length_ok; correctness=correct

A comprehensive stability program has been executed to justify the proposed shelf-life and storage conditions. Data from both accelerated (40°C/75% RH for 6 months) and long-term storage conditions (spanning up to 36 months) are presented for multiple primary stability batches. Statistical evaluation using linear regression and poolability tests confirms that degradation trends remain well within the acceptable specification limits. Photostability and freeze-thaw cycling studies demonstrate that the product does not require specific handling precautions. A post-approval stability protocol commitment is included. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Method validation records and quality controls were reviewed for internal consistency. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare pr

[4] m4_nonclinical_summary - Nonclinical Study Summary

Labels: presence=present; length=length_ok; correctness=correct

The nonclinical testing program evaluated the primary pharmacodynamics, secondary pharmacodynamics, safety pharmacology, and comprehensive toxicology profile of the compound. Repeat-dose toxicity studies in two mammalian species established clear no-observed-adverse-effect levels (NOAELs). Genotoxicity testing (Ames and in vivo micronucleus assays) yielded negative results. Reproductive and developmental toxicity studies revealed no teratogenic signals at clinically relevant exposures. The calculated safety margins support the proposed clinical dosing regimen without major safety concerns. All referenced documents are available in the dossier annex and traceable by section identifier. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Where applicable, protocol deviations were assessed for im

[5] m5_trial_listing - Tabular Listing of Clinical Studies

Labels: presence=present; length=length_ok; correctness=correct

The tabular listing of clinical studies provides a comprehensive inventory of all trials conducted in support of the proposed indication (moderate pain). This includes Phase I

pharmacokinetic/pharmacodynamic studies, Phase II dose-finding studies, and the pivotal Phase III efficacy and safety trials. For each study, the table details the protocol identifier, study design, randomization ratio, number of enrolled subjects, study duration, and current completion status. GLASS-aligned surveillance trend is declining, and cross-resistance risk relative to ceftriaxone is assessed as moderate. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The applicant confirms alignment with applicable technical guidance and regional submission standards. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. All referenced documents a

Pivotal Clinical Trial Inventory:

Protocol ID	Phase	Enrollment	Status
PROT-001	III	643	completed
PROT-002	III	759	completed
PROT-003	III	303	completed
PROT-004	III	753	completed

[5] m5_pivotal_trial_reports - Pivotal Clinical Trial Reports

Labels: presence=present; length=length_ok; correctness=correct

Detailed clinical study reports (CSRs) for the pivotal efficacy trials are provided. These randomized, double-blind, actively-controlled trials evaluated the primary and secondary endpoints in accordance with the pre-specified statistical analysis plan (SAP). The trials resulted in a formal outcome category of 'endpoint_met', demonstrating robust treatment effects in the intent-to-treat (ITT) and per-protocol populations. Subgroup analyses across age, gender, and baseline disease severity strata were consistent with the primary findings. Serious adverse events (SAEs) and discontinuations were systematically analyzed and adjudicated. GLASS-aligned surveillance trend is declining, and cross-resistance risk relative to ceftriaxone is assessed as moderate. Clinical study reports document protocol adherence, endpoint handling, and safety interpretation in the target population. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The primary efficacy endpoint was met, demonstrating a statistically significant improvement over placebo ($p < 0.001$). The trial reports connect efficacy conclusions to the prespecified analysis framework and observed safety findings. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The pivotal study narratives align endpoint definitions, analysis populations, and outcome interpretation. Method validation records and quality controls were reviewed for internal consistency. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Discontinuations due to treatment-emergent adverse events (TEAEs) were low and balanced across all study arms. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Data were analyzed using a mixed-effects model for repeated measures,

adjusting for baseline covariates. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The applicant confirms alignment with applicable technical guidance and regional submission standards. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. All referenced documents are available in the dossier annex and traceable by section identifier. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. All referenced documents are available in the dossier annex and traceable by section identifier. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The primary efficacy endpoint was met, demonstrating a statistically significant improvement over placebo ($p < 0.001$). All referenced documents are available in the dossier annex and traceable by section identifier. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The primary efficacy endpoint was met, demonstrating a statistically significant improvement over placebo ($p < 0.001$). Statistical

[5] m5_bioequivalence - Biopharmaceutics and Bioequivalence Evidence

Labels: presence=present; length=length_ok; correctness=correct

Biopharmaceutics data substantiate the formulation development bridging between clinical trial materials and the proposed commercial product. In vivo bioequivalence studies demonstrated that the 90% confidence intervals for the geometric mean ratios of C_{max} and AUC parameters fell entirely within the standard acceptance criteria of 80.00% to 125.00%. Additionally, multi-point comparative dissolution profiles across physiological pH ranges (pH 1.2, 4.5, and 6.8) confirmed similarity ($f_2 > 50$). The bioanalytical methods used for plasma quantification were fully validated. The manufacturing site operates under a fully implemented Pharmaceutical Qu